

The Architecture of *Grace*

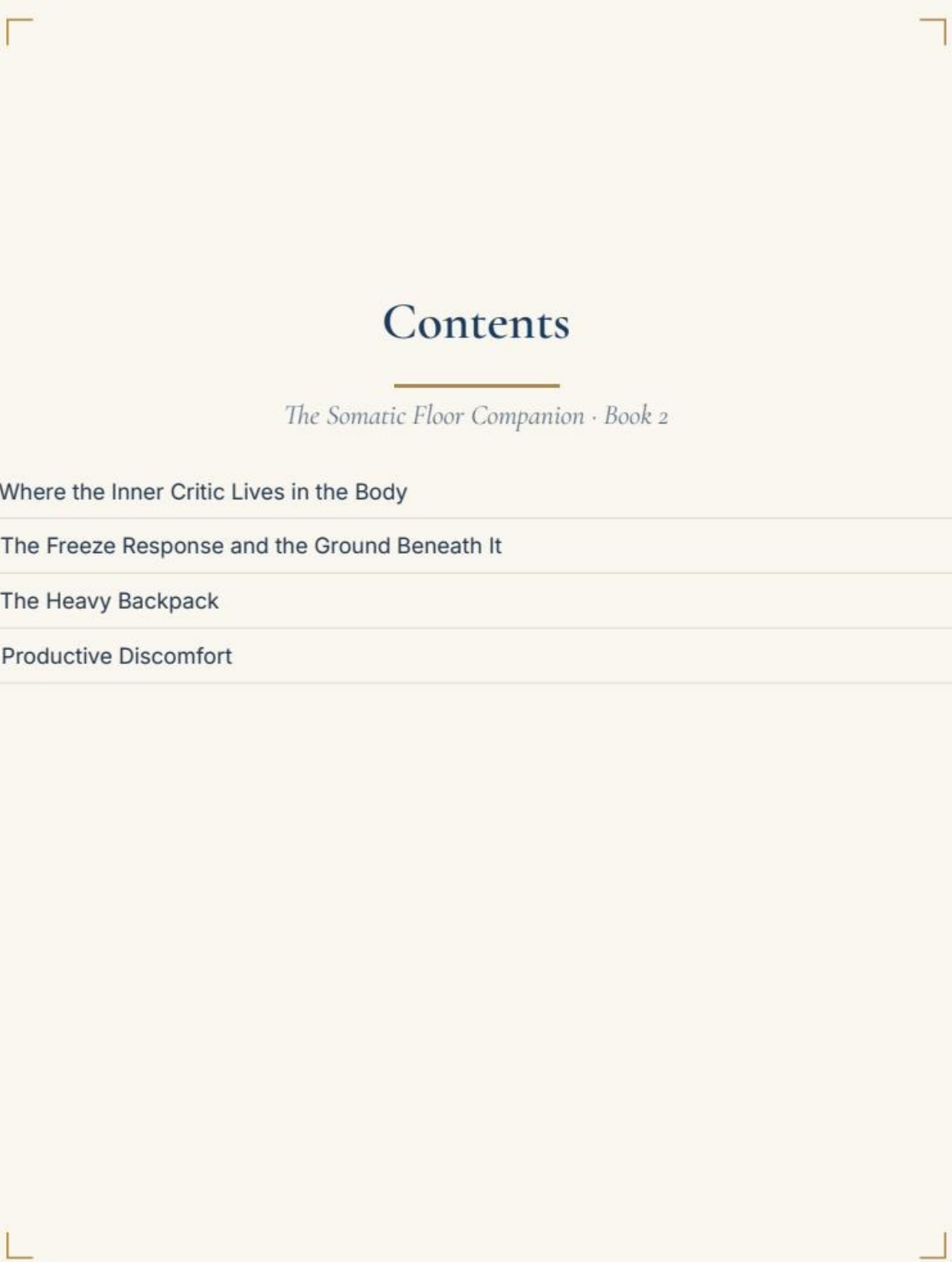
Somatic Floor & Regulation

A clinical & regulation companion to Book 2 · The Framework

GRADES 3 · 4 · 5 · COMPANION EDITION

SECULAR · CLINICAL · TRAUMA-INFORMED

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Where the Inner Critic Lives in the Body

Interoception, Regulation, and the Calm Zone

GRADES 3 – 5 · COMPANION TO U1-L4

Why This Document Exists

Book 2's Unit 1 Lesson 4 (Inner Critic vs. Inner Coach) is cognitively excellent. Deliverable 3 gives you the floor underneath it. Both are needed. Neither replaces the other.

The clinical reality every teacher and parent of a school-age child needs to understand: the Inner Critic does not announce itself first as a thought. By the time a child can hear the words "I am stupid" forming in their mind, those words have already arrived as physical sensations — a tightening in the chest, a heat in the face, a clenching in the gut, a flatness in the limbs. The thought is the second wave. The body is the first.

For neurotypical children, this body-first wave is usually below conscious awareness and the cognitive intervention can still land. They feel a flicker of tightness, the critical thought arrives, and an adult saying "that sounded like the Inner Critic — what would the Inner Coach say?" can interrupt the spiral while the nervous system is still inside the window of tolerance.

For autistic children, complex-trauma children, and highly anxious children, this is often not true. The first wave is larger, faster, and more total. The nervous system can move from baseline to full sympathetic activation — flight, fight, fawn — or into dorsal collapse — freeze, shutdown — in under three seconds. Once there, **the cognitive intervention is not available.** The pre-frontal cortex is offline. The child cannot reason. They cannot remember the Inner Coach questions. They cannot hold the cognitive reframe in their working memory because their working memory has gone elsewhere. They are not refusing — they are temporarily unable.

WHAT THIS MEANS PRACTICALLY

Before the Inner Critic / Inner Coach lesson can do its work, the child must be inside the window of tolerance. If they are not, no amount of teaching helps. The lesson must therefore include, as a structural prerequisite, the somatic skills that let a child notice the body-first wave and return to the Calm Zone before the cognitive intervention is attempted.

This document provides those skills. They are not optional add-ons for the autistic or anxious child. They are the operating floor for every child. The neurotypical child needs them less often, but when they need them, they need exactly the same ones.

THE CLINICAL FRAMEWORK

This document is grounded in established clinical literature: Polyvagal Theory (Porges), the Window of Tolerance model (Siegel), interoception research (Mahler, Kelly, Garfinkel), trauma-informed practice (van der Kolk, Perry), and Brown's shame research as it relates to embodiment. None of the protocols here are speculative. All have been used in clinical and educational settings for decades. This document organizes them specifically around the Inner Critic activation pattern that Book 2 addresses.

THE GUIDING PRINCIPLE OF THIS DOCUMENT

Regulation precedes cognition. The body must be calm enough for the mind to listen. **We do not skip steps.** The Inner Coach work cannot land on a nervous system that has not yet returned to baseline. The path from "Inner Critic loud" to "Inner Coach available" runs through the body.

The Three-Zone Model

Window of Tolerance · adapted for Grade 3-5 vocabulary

Book 2 already teaches the Window of Tolerance in U1-L2 (Emotions Are Messengers / Regulation Toolkit). This document extends that teaching by mapping the Inner Critic directly onto the zones. The Critic does not show up the same way in every zone — and the response that works in one zone is wrong for another. Knowing which zone the child is in is the first job.

HYPER-AROUSAL	WINDOW OF TOLERANCE	HYPO-AROUSAL
WHAT THE BODY FEELS ▶ Heart racing ▶ Chest tight or fluttery ▶ Hot face, hot hands ▶ Jumpy belly ▶ Shaky hands ▶ Hard to sit still ▶ Thoughts racing	WHAT THE BODY FEELS ▶ Breath is even ▶ Heart is steady ▶ Body feels safe ▶ Can listen to others ▶ Can think clearly ▶ Can feel feelings without drowning in them	WHAT THE BODY FEELS ▶ Heavy arms and legs ▶ Sunk feeling in chest ▶ Far away from the room ▶ Voice is small or gone ▶ Tired, blank, foggy ▶ Can't seem to care ▶ Eyes drop, gaze far away
HOW THE CRITIC SHOWS UP Loud, fast, attacking. "You're going to fail. Everyone is watching. You're stupid for not knowing this." The Critic acts like an emergency.	HOW THE CRITIC SHOWS UP Present, but not in charge. You can hear the Critic AND the Coach. You can choose which one to listen to. This is where the lesson can work.	HOW THE CRITIC SHOWS UP Quiet, heavy, hopeless. "Why even try. You're nothing. Nothing matters. There's no point." The Critic acts like a fact.
WHAT WORKS HERE Slow the body down. Long exhale breath. Cold water. Feet on the floor. Hand on chest. Slow, low voice from an adult. DO NOT teach the lesson now. Co-regulate first.	WHAT WORKS HERE Teach. Reflect. Practice. Inner Critic / Inner Coach work. Worksheet practice. Pastoral or counseling conversation. The whole lesson belongs HERE.	WHAT WORKS HERE Wake the body up. Gentle movement. Stand up. Walk. Cool water on the face. Strong taste (sour candy). Bright light. A trusted person nearby. DO NOT teach the lesson now. Re-engage the body first.

THE KEY DISTINCTION

You cannot teach a child the Inner Coach when they are in Too Much or Too Little. You can only help them **come back**. The lesson lives in the Calm Zone. The somatic protocols are how a child gets there. Co-regulation precedes self-regulation. Both precede the cognitive lesson.

The Inner Critic Body Map

Where the Critic shows up before the words arrive

This is the centerpiece tool of this document. Children — and adults — often experience the Inner Critic as cognitive: a voice in the head saying mean words. But the voice does not come first. The body comes first. By the time a child can articulate the critical thought, the nervous system has already been activated for several seconds.

Teaching a child to notice the body signal **before** the words arrive gives them a second of warning that changes everything. In that second, the Inner Coach is still reachable. After the words land, the Critic has usually already done its work.

WHERE THE INNER CRITIC LIVES IN MY BODY

Often — but not always. Bodies vary. Trust your own.

▶ LOUD THOUGHTS ▶ SPINNING IDEAS ▶ BUZZING ▶ PRESSURE OR ACHE ▶ FOGGY / CAN'T FOCUS	▶ LUMP ▶ TIGHTNESS ▶ HARD TO SWALLOW ▶ VOICE SMALL OR GONE	▶ FAST HEART ▶ TIGHT BAND ▶ HOT OR BURNING ▶ EMPTY / SUNK FEELING ▶ HARD TO BREATHE DEEP	▶ KNOTS OR TWISTING ▶ COLD PIT ▶ HOLLOW / EMPTY ▶ NAUSEOUS ▶ HEAVY STONE FEELING
FACE ▶ Hot cheeks ▶ Tight jaw ▶ Wet eyes ▶ Gaze dropping ▶ Flat / no expression	SHOULDERS & BACK ▶ Pulled up to ears ▶ Curving forward ▶ Heavy backpack feeling ▶ Tight neck	HANDS & ARMS ▶ Shaking ▶ Tingling or numb ▶ Clenched fists ▶ Sweating ▶ Heavy / can't move	LEGS & FEET ▶ Bouncing knees ▶ Wanting to run ▶ Heavy / frozen ▶ Curl-up feeling ▶ Want to hide under desk

IT IS IN MY WHOLE BODY AT ONCE. THE CRITIC FEELS LIKE IT FILLS ME UP. WHEN THIS HAPPENS, I NEED THE MOST HELP. A GROWN-UP TO HELP ME COME BACK. THIS IS OKAY TO ASK FOR.

I CANNOT TELL WHERE IT IS IN MY BODY. THIS IS A REAL, HONEST ANSWER. MY BODY JUST WORKS DIFFERENTLY. NOTICING THE NOT-SURE IS STILL NOTICING. THE GROWN-UPS IN THIS ROOM WILL HELP ME ANYWAY.

AUTISM-SPECIFIC NOTE · INTEROCEPTION

Many autistic students have **interoception differences** — the body-sense that delivers feeling-data may be quieter, delayed, or differently mapped than this chart assumes. A child who says "I don't feel it anywhere" is not lying or refusing. They are telling you the truth about how their body works. **For these children, the body map is not yet the right tool.** Start instead with external indicators they can notice: posture (am I curled up?), voice (is my voice gone?), eyes (where am I looking?), hands (what are they doing right now?). These external observations bypass interoception and give the child somatic data they CAN access.

The Four Calm-Zone Return Protocols

Scripted interventions for the four most common Inner Critic activation patterns

These four protocols cover roughly 90% of Inner Critic activation moments in Grades 3–5. Each protocol assumes the child is currently OUT of the Calm Zone and cognitive work is not yet available. Each is designed to be deliverable in under 5 minutes, by any adult, without specialized training, in the actual settings where these moments happen (classroom, hallway, home).

Memorize the names. Carry the cards. When the moment comes, you will not have time to look up the right tool. The right tool has to already be in your hands.

PROTOCOL 1

· FOR TOO MUCH (HYPER-AROUSAL) **The Long Exhale Use when:** Heart racing, hot face, jumpy belly, shaky hands, hard to sit still, thoughts racing, voice loud and fast, Inner Critic acting like an emergency. **Why this works:** A long exhale (longer than the inhale) directly activates the parasympathetic nervous system through the vagus nerve. This is not a metaphor — it is a measurable physiological mechanism. Within 60–90 seconds of slow exhales, heart rate drops and the body begins to leave fight/flight. **THE PROTOCOL · STEP BY STEP 1.** Lower your own voice and slow down. Match the child's energy briefly to attune, then slowly lead downward. "I can see something is happening. Let's slow down together." **2.** Put both feet on the floor. Yours and theirs. Say so out loud. "Both feet on the floor. Like this." Model it visibly. The child does not have to comply — your modeling alone begins the co-regulation. **3.** Breathe with them. In for 4. OUT for 6 or 8. The longer exhale is the active ingredient. Do not just say "take a deep breath" — count out loud, slowly. "In, two, three, four. Out, two, three, four, five, six." Three rounds minimum. **4.** Name what you see, without judgment. "Your shoulders just came down a little. Your breathing slowed. That's your body coming back. You did that." **5.** DO NOT proceed to the cognitive lesson until at least 90 seconds of even breathing. If the child is still escalated, do another round. The clock is not the boss; the body is. **What to say (verbatim): "The Inner Critic got loud. I can see it. Your body got loud first. That's how it works. We don't have to fight the words right now. We just need to slow your body down. Then we can talk about the words. Breathe with me. In, two, three, four. Out, two, three, four, five, six."**

PROTOCOL 2

· FOR THOUGHTS RACING / DISSOCIATION **The 5-Senses Ground Use when:** Child is "far away," thoughts spinning, can't focus, eyes glazed, voice flat, or describes feeling "not real" / "not in the room." Also useful when the Inner Critic is repetitive and looping. **Why this works:** Naming sensory data out loud forces the prefrontal cortex to engage and pulls the nervous system out of dissociation by anchoring it to the present moment. Research on the affect-labeling effect (Lieberman et al.) shows measurable amygdala reduction when feelings and sensations are put into words. **THE PROTOCOL · STEP BY STEP 1.** Sit at the child's level. Speak gently. "Let's look around for a minute. I'm going to count with you." **2.** Walk through the five senses, in order. Have the child name aloud (or point to, for non-verbal moments): ► **FIVE things I can see ► FOUR things I can touch ► THREE things I can hear ► TWO things I can smell ► ONE thing I can taste** **3.** Let the answers be small. "The blue eraser. The corner of my desk. The ceiling tile." These are correct. Specificity helps. Do not require profound answers. **4.** End with: "You came back into the room. That was hard. Your body did good work."

PROTOCOL 3

· FOR TOO LITTLE (HYPO-AROUSAL / SHUTDOWN) **The Gentle Wake-Up Use when:** Child has gone quiet, gaze is far away, body is heavy, voice is gone, head is on the desk, shoulders curved forward — the Inner Critic is acting like a fact rather than an emergency. "I'm nothing. Nothing matters. There's no point." **Why this works:** Hypo-arousal is a dorsal vagal shutdown — the nervous system has determined the situation is unmanageable and collapsed. Gentle sensory stimulation re-engages the system without triggering full sympathetic activation. The goal is upward toward the Calm Zone, not jumping into hyper-arousal. **Special caution:** Too Little is HARDER to recognize than Too Much because the child looks compliant, quiet, well-behaved. Many teachers miss it. The Marcus scenario from Book 2's regulation strand (R1-B) is exactly this. Watch for it. **THE PROTOCOL · STEP BY STEP** **1.** Approach gently. Sit beside, not in front of. Do not crouch suddenly into their visual field. "Hey. I'm right here." **2.** Offer ONE sensory input. NOT all at once. Choose: a cool drink of water. A cold tile against the wrist. A bright light turned on. Standing up. A walk to the water fountain. A piece of sour candy. Strong-smelling lotion. **3.** Give choices, not commands. "Would water help, or standing up?" The autonomy itself is part of the wake-up. **4.** Gentle movement helps most: "Let's walk to the window together." Even 20 feet of walking activates the system upward without flooding it. **5.** Once the child's voice returns, even faintly — they have come up. Now you can name it: "You came back. The heaviness is real. We can talk about it when you're ready." **What you do NOT do:** Do not say "cheer up" or "it's not that bad." Do not try to talk them out of the heavy feeling. Do not require eye contact. The shutdown is the body's last-resort protection. You honor it by helping the body decide it is safe to come back — not by demanding the child override it.

PROTOCOL 4

· FOR STUDENT SELF-USE (THE GOAL) **The Three-Part Naming Use when:** The child is INSIDE the Calm Zone but feels the Inner Critic starting to arrive. The body signal is detectable but the activation has not yet taken over. This is the protocol that, over time, becomes the child's primary self-regulation tool. **Why this works:** Naming a sensation reduces its power. Functional MRI research (Lieberman et al.) shows that putting feelings into words measurably reduces amygdala activation. The three steps match Book 2's existing "Feelings Are Messengers" structure (where do I feel it, what is its name, what does it need). **THE PROTOCOL · THREE STEPS THE STUDENT LEARNS** **STEP 1 · WHERE IS IT?** "Where in my body is the Critic right now? My chest? My belly? My throat? My head?" **STEP 2 · WHAT IS IT DOING?** "Is it tight, hot, fast, heavy, jumpy, sunk, fluttery, frozen? What does it feel like?" Naming the sensation as a sensation (not as a verdict about the self) is the move. "My chest is tight" is data. "I'm a failure" is the Critic putting words on top of the data. **STEP 3 · WHAT DOES MY BODY NEED?** "Slow breath? Cold water? Feet on the floor? A walk? Quiet? A person nearby?" **The goal:** By Grade 5, a child can run this whole sequence in 30 seconds, in their seat, during a test or a hard moment, without an adult. That is the destination. We are building the muscle.

The Autism-Specific Layer

Sensory accommodation calibrations for autistic students across the support-needs spectrum

Everything in this document so far applies to all students. This section adds layers specifically calibrated for autistic learners. Use this section in addition to, not instead of, what comes before.

A reminder: support needs are not static. A student who looks like Level 1 on most days may need Level 2 or 3 support during a fire drill, an unexpected schedule change, or after an Inner Critic spike. The protocols should be available to any student who needs them in the moment — not assigned permanently by diagnosis.

FIVE AUTISM-SPECIFIC CALIBRATIONS

FOR AN AUTISTIC CHILD, SENSORY INPUT IS OFTEN THE INNER CRITIC'S AMPLIFIER. FLUORESCENT LIGHTING, SCRATCHY CLOTHING, THE SMELL OF CAFETERIA FOOD, THE VOLUME OF THE HALLWAY BETWEEN CLASSES — THESE CAN DRIVE ACTIVATION INDEPENDENTLY OF ANY COGNITIVE TRIGGER. BEFORE RUNNING ANY PROTOCOL WITH AN AUTISTIC CHILD, AND REDUCE INPUT IF POSSIBLE. DIM LIGHTS. MOVE TO A QUIETER LOCATION. OFFER NOISE-CANCELING HEADPHONES. LOWER YOUR VOICE FURTHER THAN FEELS NECESSARY.

WHEN AN AUTISTIC CHILD ROCKS, HAND-FLAPS, VOCALIZES, TAPS, PACES, OR REPEATS A PHRASE — THEY ARE ALREADY RUNNING A REGULATION PROTOCOL. IT IS HELPING. YEARS OF BEHAVIORAL INTERVENTION HAVE TAUGHT MANY AUTISTIC CHILDREN TO SUPPRESS VISIBLE STIMMING, OFTEN AT GREAT REGULATORY COST. IF A CHILD STIMS DURING THE INNER CRITIC MOMENT, NAME IT AS OKAY: "

MANY AUTISTIC STUDENTS CANNOT GENERATE A FRESH INNER COACH STATEMENT IN THE MIDDLE OF ACTIVATION — BUT THEY CAN DEPLOY A MEMORIZED SCRIPT. A CHILD WHO RECITES THE THREE INNER COACH QUESTIONS VERBATIM, OR WHO REPEATS A CALMING PHRASE IN THE SAME TONE EVERY TIME, IS DOING THE LESSON. THE MEMORIZED FORM IS THE FUNCTIONAL FORM. PROVIDE THE SCRIPT IN ADVANCE. PRINT IT ON A CARD. LET THE CHILD CARRY IT. "WHEN THE CRITIC IS LOUD, YOU CAN SAY THE WORDS ON THIS CARD. YOU DON'T HAVE TO MAKE UP NEW ONES." THE CARD MIGHT SAY: THIS IS HARD. THE CRITIC IS LOUD. MY BODY NEEDS SLOW BREATH. I AM SAFE. THE SCRIPT IS THE INNER COACH FOR THIS CHILD. THAT IS NOT A WORKAROUND; THAT IS A DEVELOPMENTALLY HONEST FORM OF THE SKILL.

OPEN-ENDED CHOICES ("WHAT WOULD HELP RIGHT NOW?") DEMAND EXECUTIVE FUNCTION THAT THE ACTIVATED NERVOUS SYSTEM HAS TEMPORARILY LOST. REPLACE WITH CONCRETE, SEQUENCED OFFERS: " " " OR USE A VISUAL CHOICE BOARD: PICTURES OF HUG · BREAK · QUIET · PERSON · MOVE. POINT-TAP IS A FULL RESPONSE. A CHILD WHO CAN POINT TO "QUIET" ON A CHOICE BOARD HAS JUST REGULATED. THE POINTING IS THE WORK.

FOR AUTISTIC STUDENTS WITH MODERATE SUPPORT NEEDS, SELF-REGULATION MAY NOT BE DEVELOPMENTALLY AVAILABLE FOR YEARS — PERHAPS NOT UNTIL LATE ADOLESCENCE. THE ADULT IS THE REGULATOR. THAT IS NOT A FAILURE OF THE CURRICULUM OR THE CHILD; IT IS HOW NERVOUS SYSTEMS DEVELOP. WHAT THIS LOOKS LIKE IN PRACTICE: THE ADULT RUNS THE PROTOCOL OUT LOUD, FOR*** THE CHILD, WITH THE CHILD PRESENT, EVERY TIME. OVER YEARS, THE CHILD LEARNS THE SEQUENCE BY HEARING IT FROM YOU IN THEIR HARDEST MOMENTS. ONE DAY, IN ADOLESCENCE OR ADULTHOOD, THE SEQUENCE RUNS IN THEIR OWN HEAD, IN YOUR VOICE. THAT IS THE ARCHITECTURE. FAITHFULNESS OVER TIME MATTERS MORE THAN ANY SINGLE INTERVENTION. SHOW UP THE THIRD TIME. THE FIRST TWO WERE THE PRACTICE.*

A NOTE ABOUT DIGNITY

Autistic students experience the world in their autistic bodies. That is not a malfunction to be corrected.

The interoception differences, the sensory sensitivities, the patterns of regulation that look unusual to neurotypical observers — these are part of how a particular person experiences being human. The protocols in this document are not designed to make autistic children less autistic. They are designed to give every child the somatic skills to access the Calm Zone where learning, growth, and relationship can happen. Same Window of Tolerance. Same neurological architecture. Different sensory calibrations.

Grade-Band Scripts

Calibrated language for Grades 3, 4, and 5

The protocols above use Grade 4–5 language as the baseline. Below are calibrations for Grade 3 (slightly more concrete) and Grade 5 (slightly more sophisticated).

GRADE 3

Concrete. Brief. Body-first language. **WHEN INTRODUCING THE ZONES** ▶ "Your body has three zones. Too much — that's when your heart goes fast and your hands shake. Calm zone — that's when you feel okay. Too little — that's when you feel heavy and small and far away. They are all real. None of them are bad. The trick is knowing which one you are in." **WHEN TEACHING THE BREATH** ▶ "Smell the flower. Blow out the candle. Slow. Slow. Like that."** (The flower-and-candle is the most reliable Grade 3 breath teaching.)

GRADE 4

Use the protocol language above as written. This is the baseline calibration. **ADDITIONS FOR GRADE 4** ▶ Introduce the word "nervous system." "Your body has a nervous system — it's how your body decides if it's safe or in danger. When the Critic gets loud, your nervous system thinks there's danger, even though there isn't. The breathing tells it: actually, we are safe." ▶ ***Introduce the Three-Part Naming. Walk through it as a class. Have students practice with a small, low-stakes example before the real moment arrives.*

GRADE 5

Push toward specificity and autonomy. Students at this level can take ownership of their own regulation. **ADDITIONS FOR GRADE 5** ▶ Push past "breathe" to specificity: "At what moment will you breathe? How will you know you need to? What is the signal in YOUR body that the Critic is starting? Name it specifically." ▶ **Introduce the concept that the body's signal is information, not weakness: "Your body noticing the Critic is your body doing its job. The signal is not the enemy. The signal is the warning system. Thank the signal. Then run your protocol."**

Closing Word for the Teacher

You will not run all four protocols every day. Most days, the children will be in the Calm Zone and the lesson will work as written. But there will be days — and there will be specific children — where the body comes first, and you will need these tools.

When that day comes, **do not feel that you have failed to teach the lesson.** You taught the lesson. The lesson, on that day, for that child, was the protocol. The Inner Coach work will come back next week, when the body is ready. The faithfulness of staying with a child through dysregulation IS the curriculum. Maya did not get from "could not look up" to "looking up" through worksheets. She got there because someone stayed with her, faithfully, every day, for a month.

Your own regulation matters more than any tool in this document. A dysregulated adult cannot co-regulate a dysregulated child. If you find yourself escalating with a child, the first protocol is your own: feet on the floor, slow exhale, name the body sensation. Then return. The repair, when you don't get it right, IS the lesson. The children will learn from how you come back as much as from any chart on the wall.

THE THREE THINGS TO REMEMBER

1. Regulation precedes cognition. The body must be in the Calm Zone for the lesson to land. Slow the body first. **2. Co-regulation precedes self-regulation.** You are the Inner Coach until the child can build their own. For some children, that is years of patient presence. That is the work. **3. The body is not the enemy.** Slow breath, cold water, a walk to the window — these are real interventions, grounded in measurable physiology. The nervous system is on your side. Honor it.

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